



Before the Fall: Connecting High-Risk Adults to Prevention Before Emergency Department Presentation

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WHY IT MATTERS

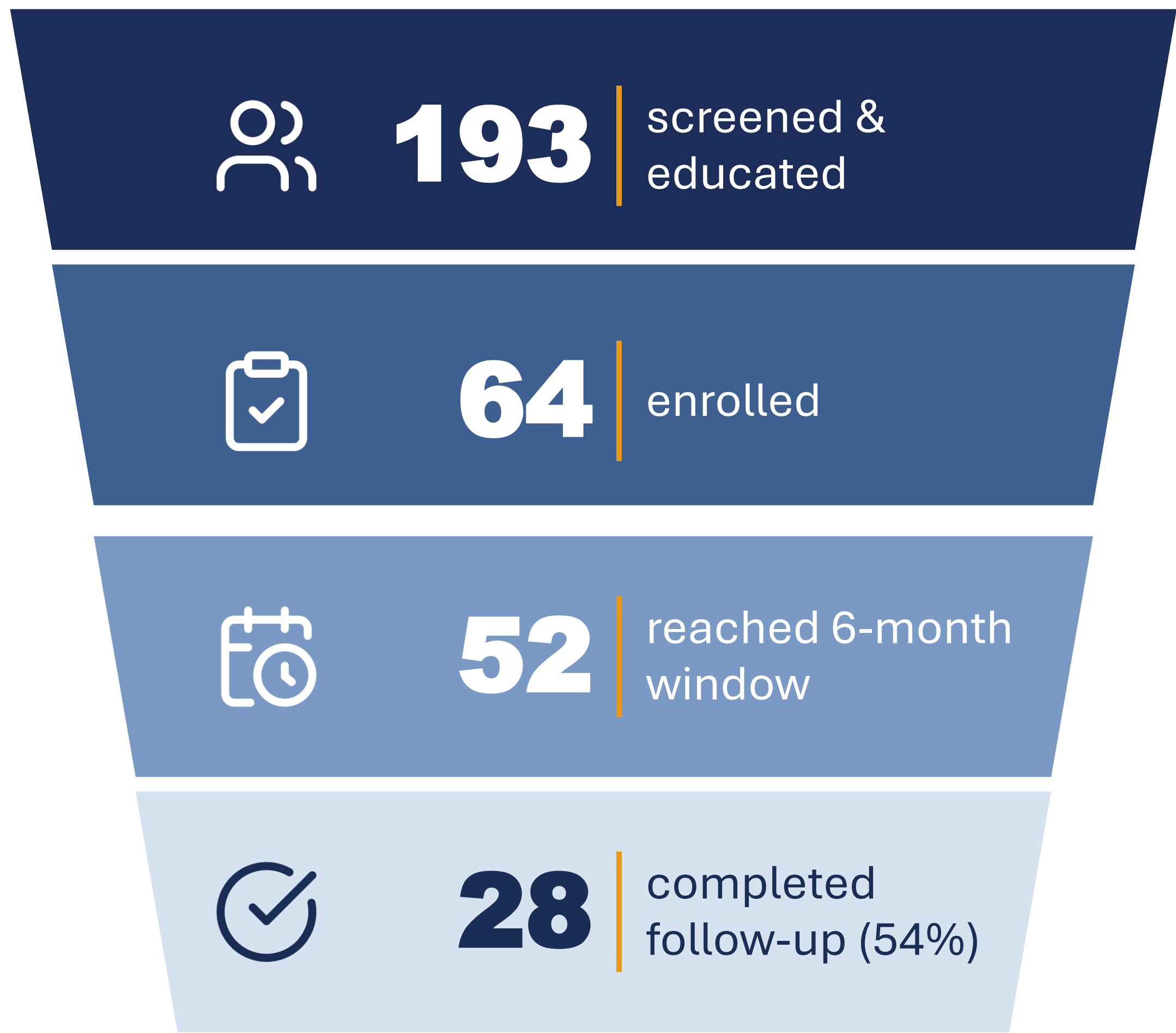
- Unintentional falls drive **~8.9 million U.S. emergency department visits** each year.¹
- The CDC's **Stopping Elderly Accidents, Deaths, and Injuries (STEADI)** is an evidence-based screen–assess–intervene framework.³
- Multifactorial and multiple-component interventions **can reduce fall rates**.²
- Evidence-based exercise **programs reduce falls**.⁵
- Validated prevention exists, but **delivery in primary care remains inconsistent**.⁴

PILOT OBJECTIVES

- Reach** high-risk community-dwelling adults.
- Characterize** fall risk, concern, and prior prevention engagement.
- Intervene** with motivational interviewing, tailored education, and community linkage.
- Assess** 6-month prevention engagement, acute-care utilization, and falls.
- Evaluate** feasibility to inform a hybrid type 1 trial.

PARTICIPANTS

5 events | 2023–2026 | Rural & metro
Mean age 73.6 ± 9.8 y | 75.0% female



FROM SCREENING TO PREVENTION

15% → 46%

reported fall-prevention class participation at 6 months
among paired respondents (n=26)

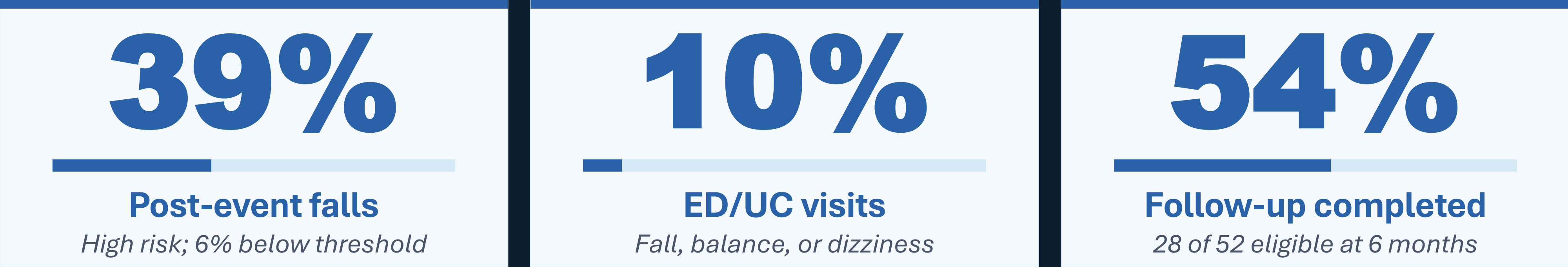
Two-thirds screened high risk, yet most had little prior prevention engagement

PILOT RESULTS AT A GLANCE

AT ENROLLMENT

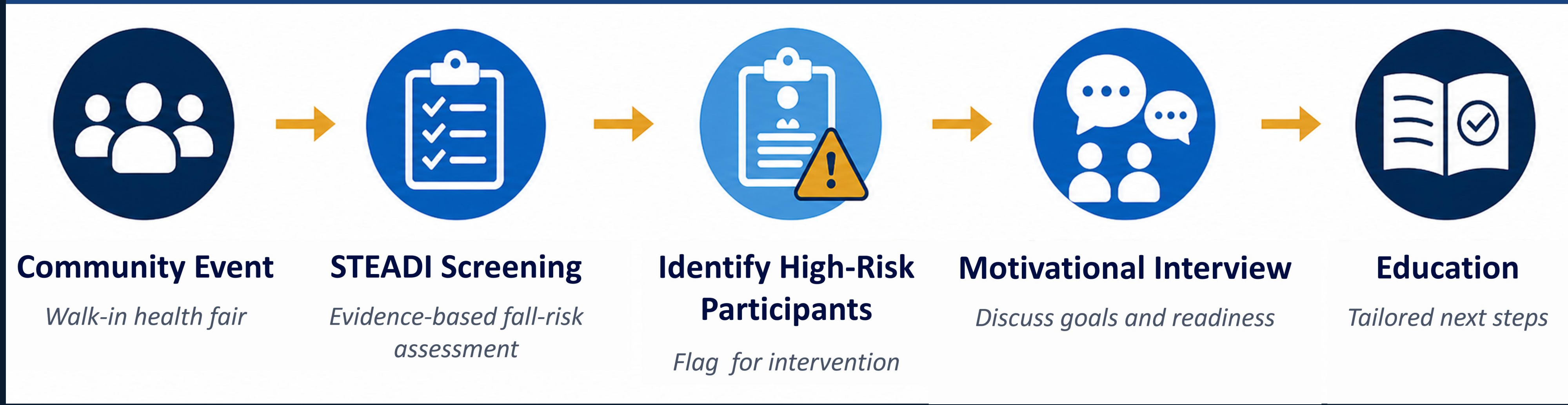


AT 6-MONTH FOLLOW-UP



IMPLEMENTATION MODEL

SCREENING-TO-INTERVENTION PATHWAY



WHAT WE FOUND

- 66.7%** screened high risk
STEADI ≥4; 42/63
- 54.7%** fell in the prior year
76.2% of high-risk participants
- 57.8%** no prior PCP discussion
- 87.5%** never attended a class
- 85.9%** moderate-to-high concern
- 15.4% → 46.2%** fall-prevention class participation
Baseline to 6 months; paired, n=26
- 39.4% vs 5.6%** post-event falls
High risk vs below; RR 7.09 (95% CI 1.01–49.9)
- 9.6%** post-event fall-related ED/UC visits

WHAT THIS MEANS

- The model reached adults with high risk, high concern, and little prior prevention engagement and **delivered individualized fall-prevention intervention**
- Prevention engagement increased** post-event
- Pilot **feasibility, targeting, and outcome-rate estimates** will inform the design of a larger trial

LIMITATIONS

- Self-selected pilot**; no comparison group.
- Limited follow-up** among eligible participants
- Fall ascertainment relied on **self-report and EHR review**; out-of-system events may be **missed**
- STEADI validated for **ages ≥65; 11 were younger**
- English-only consent **limited enrollment**

FUTURE DIRECTIONS

- Continue and expand** community fall-prevention events and community-partner linkage
- Geospatially target** rural, high-burden Oregon communities.
- Hybrid Type 1 trial** testing prevention uptake, falls, and fall-related acute-care utilization.

References:

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Acknowledgments:

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